



My Courses (<https://learningcenter.hfsa.org/Users/Home.aspx>)
/ **2026 AHFTC Board Certification Review - OnDemand**
(<https://learningcenter.hfsa.org/Users/UserOnlineCourse.aspx?LearningActivityID=4f%2fOexzNsorKAF1lCFotvA%3d%3d>)
/ **2026 AHFTC Board Certification Question Bank**
(<https://learningcenter.hfsa.org/Users/UserOnlineCourse.aspx?LearningActivityID=YSpCYks4EUa7JCy700Rkkw%3d%3d>)
/ **Practice Exam - 1**

Correct Answer

Next Q

Practice Exam - 1

Question 58 of 117



You admit an 83-year-old widow to the hospital for acute on chronic HFrEF and atrial fibrillation. She has been hospitalized 3 times in the past 6 months for volume overload, and one of those hospitalizations was associated with renal failure requiring temporary dialysis. She is home-bound and has significant difficulty managing her activities of daily living. During hospitalization, she expresses a desire to be made comfortable, avoid additional hospitalizations, and not be resuscitated. Her adult daughter and son who accompany her to the clinic visit strongly advocate for all measures to be taken to extend her life. You offer a specialist palliative care consultation. Which of the following is a benefit of palliative care intervention:

- A. Palliative care intervention will resolve the conflict between the patient and her family
-  B. Heart failure palliative care interventions have been shown to reduce the number of subsequent heart failure hospitalizations and prolong survival
- C. Heart failure palliative care interventions have been shown to reduce the number of subsequent heart failure hospitalizations but not prolong survival
-  D. Heart failure palliative care interventions have been demonstrated to improve quality of life

Commentary

Correct Answer: D. Heart failure palliative care interventions have been demonstrated to improve quality of life

Rationale: The PAL-HF trial demonstrated that specialist palliative care interventions in patients with advanced heart failure improved quality of life, including symptom burden, anxiety, depression, and spiritual well-being (primary endpoint), without demonstrating reductions in hospitalizations or mortality.

Answer A: Palliative care intervention does not necessarily resolve family conflict, but it may improve communication, facilitate shared decision-making, and help align treatment with patient goals.

Answer B: Palliative care involvement has not been shown to prolong survival in patients with heart failure, although survival benefit

has been demonstrated in some malignancies.

Answer C: Although some studies suggest palliative care may reduce healthcare utilization, heart failure palliative care interventions have not consistently been shown to reduce subsequent hospitalizations, and this is not an established primary benefit.

Powered by Oasis.



(<https://www.oasis-lms.com/>).